



ChiroPillar
STRENGTH IN ALIGNMENT

ChiroPillar Digital

Back-Pain & Telehealth App — Product, Monetization & Insurance Deep Dive

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Free → \$9/month → \$74/year, with telehealth and local doctor/chiropractor matching

Contents

1. Executive Summary

ChiroPillar Digital is a free-to-start mobile app that helps people understand and manage their back (and broader musculoskeletal) pain with an AI-guided assessment and exercise program — and, when they need a human, matches them to a telehealth visit or a local ChiroPillar chiropractor/physician. It is the consumer front door to the clinic roll-up: a low-cost, high-scale engine that captures pain sufferers early, builds trust through genuinely useful free care, and converts a meaningful share into recurring app subscribers and, more valuably, into paying in-clinic patients.

The strategic logic is that the app and the clinics make each other worth more. Pure-digital MSK companies (Hinge Health, Sword Health) are worth billions but cannot deliver hands-on adjustment or in-person care; independent clinics have the hands-on care but no scalable digital acquisition funnel or recurring digital revenue. ChiroPillar can own both — a defensible position neither side can easily copy.

Three revenue layers: (1) app subscriptions (Free → \$9/mo → \$74/yr) as recurring MRR; (2) reimbursable clinical services — telehealth consults and remote therapeutic monitoring (RTM) billed through the clinics/providers; and (3) the highest-value layer, in-clinic patient conversion, where an app user who books a ChiroPillar visit becomes a multi-hundred-to-thousand-dollar lifetime patient.

Bottom line: the app is cheap to scale, improves clinical outcomes (AI exercise programs show 50–68% pain reduction in studies), creates recurring revenue, and — critically — feeds patients into the clinics that drive the platform's enterprise value and exit multiple.

2. Market Opportunity

Back pain is one of the most common and costly health complaints in the world, and the digital tools to manage it are growing fast. The digital musculoskeletal (MSK) care market is estimated at roughly \$5–10B in 2025 and is projected to reach ~\$11.6–16B by 2030–2032, growing ~17–18% per year. Online/virtual consultations are already the largest segment, and the shift toward telehealth, remote monitoring, and value-based care is accelerating.

The category has produced large winners, which both validates the model and shows the gap we can exploit:

Player	Scale / value (2025)	Model & gap
Hinge Health	~\$3B market cap at May 2025 IPO; FY25 free cash flow ~\$180M	Sells to self-insured employers; engagement-based pricing. No in-person/hands-on care.
Sword Health	~\$4B valuation (June 2025)	Therapist-led, sensor-based digital PT; acquired Kaia Health for \$285M. Employer/payer channel; no clinics.
Kaia Health	Acquired by Sword (\$285M)	Camera/computer-vision exercise coaching. Now absorbed.
ChiroPillar Digital	New entrant	Direct-to-consumer freemium + telehealth + owned local clinics. The hybrid the incumbents can't easily

Player	Scale / value (2025)	Model & gap
		build.

The wedge: incumbents win the employer contract but hand the patient nothing local when they need real hands-on treatment. ChiroPillar starts with the consumer directly, is free to begin, and can escalate to a telehealth visit or a nearby ChiroPillar clinic — closing the loop from app to adjustment table.

3. The Product

A simple, friendly app (fronted by the ChiroPillar brand and mascot) that meets people at the moment their back hurts and walks them from self-help to professional care as needed.

3.1 Core flow

1. Assess — a short AI-guided intake screens symptoms, flags red flags (e.g., signs needing urgent medical review), and classifies the likely problem.
2. Guide — a personalized daily exercise and education program adapts as the user logs pain and progress.
3. Connect — one tap to a telehealth consult or to book an in-person visit with a local ChiroPillar chiropractor or partner physician.
4. Monitor — for appropriate patients, remote therapeutic monitoring (RTM) tracks adherence and outcomes between visits — and is reimbursable (see Section 6).

3.2 Pricing tiers

Tier	Price	What's included
Free	\$0	AI back assessment, red-flag screening, starter exercise library, pain/progress tracking, educational content, clinic locator.
Plus (monthly)	\$9 / mo	Full personalized AI program, unlimited exercises & content, in-app messaging, progress insights, discounted telehealth, priority clinic booking.
Plus (annual)	\$74 / yr	Everything in Plus at ~31% off vs. monthly (best value; improves retention & cash flow).
Clinical add-ons	Billed / cash	Telehealth consults and RTM via providers (often insurance-reimbursable); in-clinic care via ChiroPillar locations.

Why this structure: Free removes all friction and maximizes top-of-funnel; \$9/mo is an easy consumer price point for a daily-use health tool; the \$74 annual plan (vs. \$108 at monthly) pulls users into a sticky, prepaid year that smooths cash flow and lifts lifetime value. The clinical layer is where insurance and high-value patient relationships enter.

4. AI & Clinical Evidence

The AI does three jobs: triage (classify the problem and screen for red flags), personalization (compose and adapt an exercise program to the individual), and escalation (decide when a human clinician should step in). This keeps the free tier genuinely helpful while routing the right patients to billable care.

The evidence base for app-based, AI-guided exercise in back pain is encouraging. Published studies of digital exercise programs report average pain reductions in the range of roughly 50–68% over 8–12 weeks, with a large cohort showing ~68% average pain improvement and ~79% of completers achieving a clinically meaningful change; controlled studies show reduced disability scores versus standard group therapy. This matters commercially as well as clinically: better outcomes drive retention, word-of-mouth, and credibility with payers.

Guardrails: the AI is a triage and coaching tool, not a diagnostician. Red-flag screening must escalate to a licensed provider; all clinical decisions and any billable services run through credentialed chiropractors/physicians. This protects patients and keeps the model compliant.

5. Monetization & Unit Economics

Three stacked revenue engines, in order of scale vs. value:

- **Subscriptions (high volume, modest ARPU):** recurring MRR from Plus users.
- **Reimbursable clinical services (mid volume, real margin):** telehealth + RTM billed through providers.
- **In-clinic conversion (lower volume, highest value):** app users who become paying ChiroPillar patients — the real prize.

5.1 Conversion benchmarks

Freemium apps typically convert ~2–5% of free users to paid; health & fitness apps that use a free trial convert far higher (median ~40%, top performers ~68%). A blended funnel using a generous free tier plus targeted trial offers can realistically target the mid-single-digits to low-teens on paid conversion as the product matures.

5.2 Illustrative MRR model (Year 1, conservative)

Assumption / output	Value
Free downloads (Year 1)	100,000
Paid conversion	4% → 4,000 subscribers
Blended ARPU	~\$8 / month
Monthly recurring revenue (MRR)	~\$32,000
Subscription ARR	~\$384,000
App users booking a clinic visit (3%)	~3,000 patients
Avg patient value to clinic (illustrative)	\$800–\$1,500

Assumption / output	Value
Clinic revenue influenced by app	~\$2.4M–\$4.5M

Read this carefully: the subscription ARR (~\$384K) is nice recurring revenue, but the in-clinic conversion line (millions) is where the app earns its keep. The app's true ROI is as the cheapest, most scalable patient-acquisition channel the clinics have — every dollar of clinic EBITDA it creates is then re-rated at the platform's exit multiple (see the roll-up deep dive).

6. Insurance & Reimbursement — “Can Insurance Cover It?”

Yes, in specific, well-defined ways — but it flows through licensed providers and clinics, not the consumer app subscription itself. The three reimbursable pathways:

6.1 Remote Therapeutic Monitoring (RTM)

RTM is the key reimbursement vehicle for an exercise/adherence app tied to musculoskeletal care. CMS established RTM CPT codes that providers (including physical therapists, and physicians) can bill under Medicare and many commercial plans:

CPT code	Approx. reimbursement	What it covers
98975	~\$20 (one-time)	Initial setup & patient education (requires ≥2 days monitoring).
98977	Device/data code	MSK monitoring data supply, 16–30 days in a 30-day period.
98980 / 98981	~\$50 / ~\$40	Treatment management, first 20 min / each additional 20 min per month.

CMS's 2026 final rule expands RTM further (new codes for shorter monitoring periods), improving the economics for MSK programs. Practically: an app-monitored patient under a ChiroPillar provider can generate recurring monthly RTM billing on top of (or instead of) the cash subscription.

6.2 Telehealth visits

Telehealth consults with a provider are reimbursable. Medicare covers many telehealth services (current flexibilities extended into 2027 for non-behavioral care in the home), and as of 2025, 44 states plus DC have private-payer telehealth laws, with ~24 states requiring payment parity (same rate as in-person). Note the chiropractic nuance: Medicare itself only pays chiropractors for manual spinal manipulation (which is inherently in-person), so telehealth billing for the app's consults is best routed through physicians/PTs or via commercial plans and cash, with chiropractic adjustments delivered in-clinic.

6.3 Cash & membership (the dependable base)

Where insurance doesn't reach, the \$9/\$74 subscription and clinic cash memberships cover it. This blended model — reimbursable where possible, cash where not — mirrors what already works in modern chiropractic and avoids over-dependence on any single payer.

Net: market the consumer app as affordable cash (\$0/\$9/\$74); layer insurance-reimbursable RTM and telehealth for clinically appropriate patients through the providers. Build RTM billing and telehealth compliance into the clinic operations from day one.

7. Go-to-Market

- **Consumer-direct:** app-store optimization, content/SEO on back-pain topics, social, and the ChiroPillar brand/mascot for memorability.
- **Clinic cross-sell:** every ChiroPillar patient is offered the app for between-visit care and RTM — instant, free distribution and higher retention.
- **Local geo-targeting:** concentrate app marketing where ChiroPillar clinics exist so digital demand converts to in-person bookings.
- **Employer/payer (later):** once outcomes data accrues, pursue the same self-insured-employer channel Hinge/Sword use — but with the differentiator of local in-person care.

8. Risks & Moats

Risks

- **Regulatory:** state telehealth and licensure rules, RTM billing compliance, and corporate-practice-of-medicine constraints (use the MSO/PC structure from the roll-up plan).
- **Clinical liability:** red-flag mis-triage; mitigate with conservative escalation and licensed-provider oversight.
- **Churn:** health-app engagement decays; combat with habit features, human touchpoints, and the annual plan.
- **Competition & CAC:** well-funded incumbents; avoid head-to-head on the employer channel early and win locally first.

Moats

- **Owned in-person network:** the one thing Hinge/Sword can't replicate quickly — local hands-on care attached to the app.
- **Data flywheel:** proprietary outcomes data across digital + in-clinic improves the AI and supports payer contracts.
- **Brand + recurring revenue:** ChiroPillar brand trust plus subscription MRR raise both retention and the eventual platform multiple.

9. Roadmap (Phased)

5. MVP (0–6 mo): free assessment + exercise library + pain tracking + clinic locator; launch in ChiroPillar markets; instrument analytics.
6. Monetize (6–12 mo): introduce \$9/\$74 Plus tiers and in-app telehealth booking; begin RTM billing through clinics.

7. Personalize (12–18 mo): mature the AI program engine; add messaging and outcomes dashboards; optimize conversion funnel.
8. Scale (18–36 mo): expand alongside clinic acquisitions; accumulate outcomes evidence; pursue employer/payer contracts.

Strategic takeaway: build the app as the patient-acquisition and recurring-revenue front end of ChiroPillar. Keep it free to start, genuinely useful, and tightly wired to the clinics — so it both stands alone as a digital-health asset and multiplies the value of the roll-up.

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All market figures, conversion rates, reimbursement amounts, and the MRR model are illustrative industry benchmarks for planning. Confirm current CPT reimbursement, state telehealth/licensure rules, and chiropractic billing limits with healthcare counsel and a billing specialist before launch. Not legal, medical, or financial advice.